got rid of the bone that in our ancestors presumably made it easy to have a nice stiff penis. Instead, we rely entirely on a hydraulic pumping system, which one cannot but feel is a costly and roundabout way of doing things. And, notoriously, erection can fail—unfortunate, to say the least, for the genetic success of a male in the wild. What is the obvious remedy? A bone in the penis, of course. So why don't we evolve one? For once, biologists of the 'genetic constraints' brigade cannot cop out with 'Oh, the necessary variation just couldn't arise.' Until recently our ancestors had precisely such a bone and we have actually gone out of our way to lose it! Why?

Erection in humans is accomplished purely by pressure of blood. It is unfortunately not plausible to suggest that erection hardness is the equivalent of a doctor's blood pressure meter used by females to gauge male health. But we are not tied to the metaphor of the blood pressure meter. If, for *whatever* reason, erection failure is a sensitive early warning of certain kinds of ill health, physical or mental, a version of the theory can work. All that females need is a dependable tool for diagnosis. Doctors don't use an erection test in routine health check-ups—they prefer to ask you to stick out your tongue. But erection failure is a known early warning of diabetes and certain neurological diseases. Far more commonly it results from psychological factors—depression, anxiety, stress, overwork, loss of confidence and all that. (In nature, one might imagine males low in the 'peck order' being afflicted in this way. Some monkeys use the erect penis as a threat signal.) It is not implausible that, with natural selection refining their diagnostic skills, females could glean all sorts of clues about a male's health, and the robustness of his ability to cope with stress, from the tone and bearing of his penis. But a bone would get in the way! Anybody can grow a bone in the penis; you don't have to be particularly healthy or tough. So selection pressure from females forced males to lose the os penis, because then only genuinely healthy or strong males could present a really stiff erection and the females could make an unobstructed diagnosis.

There is a possible zone of contention here. How, it might be said, were the females who imposed the selection supposed to know whether the stiffness that they felt was bone or hydraulic pressure? After all, we began with the observation that a human erection can feel like bone. But I doubt if the females were really that easily fooled. They too were under selection, in their case not to lose bone but to gain judgement. And don't forget, the female is exposed to the very same penis when it is not erect, and the contrast is extremely striking. Bones cannot detumesce (though admittedly they can be retracted). Perhaps it is the impressive double life of the penis that guarantees the authenticity of the hydraulic advertisement.

Now to the 'stethoscope'. Consider another notorious problem of the bedroom, snoring. Today it may be just a social inconvenience. Once upon a time it could have been life or death. In the depths of a quiet night snoring can be remarkably loud. It could summon predators from far and wide to the snorer and the group among whom he is lying. Why, then, do so many people do it? Imagine a sleeping band of our ancestors in some pleistocene cave, males snoring each on a different note, females kept awake with nothing to do but listen (I suppose it is true that males snore the more). Are the males providing the females with deliberately advertised and amplified stethoscopic information? Could the precise quality and timbre of your snore be diagnostic of the health of your respiratory tract? I don't mean to suggest that people snore only when they are ill. Rather, the snore is like a radio carrier-frequency, which drones on regardless; it is a clear signal which is *modulated*, in diagnostically sensitive ways, by the condition of the nose and throat. The idea of females preferring the clear trumpet note of unobstructed bronchi over virus-blown snorts is all very well, but I confess that I find it hard to imagine females positively going for a snorer at all. Still, personal intuition is notoriously unreliable. Perhaps at least this would make a research project for an insomniac doctor. Come to think of it, she might be in a good position to test the other theory as well.

These two speculations should not be taken too seriously. They will have succeeded if they bring home the principle of Hamilton's theory about how females try to choose healthy males. Perhaps the most interesting thing about them is that they point up the link between Hamilton's